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TITLE: Revision of Potassium Monitoring Interval for CardioZen in Chronic Kidney Disease
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MEMORANDUM

To: Prescribing leads; renal services; clinical review team
From: Chair, Medication Safety Committee
Memo reference: SAFETY_DOC_008
Date: 15 August 2026
Subject: Shortening of the potassium monitoring interval - CardioZen with chronic kidney disease

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1. Why we are revising

In March of this year the Committee issued SAFETY_DOC_007, which required serum potassium to be checked at least every three months for patients receiving CardioZen with a diagnosis of chronic kidney disease. Five months of operating experience is now available.

Two findings prompt this revision. First, among patients who did develop clinically significant potassium elevation while compliant with the three-month interval, the elevation was in most cases detectable earlier than the next scheduled measurement would have caught it. Second, the interval performed least well in precisely the group at highest risk - those with the most reduced renal function - because in that group accumulation proceeds faster than the interval assumes.

The Committee's judgement is that the interval published in March is too long for this population.

2. Revised requirement

For patients receiving CardioZen who have a recorded diagnosis of chronic kidney disease, serum potassium must be checked every 60 days for the duration of therapy.

This replaces the three-month interval published in SAFETY_DOC_007 section 3. It takes effect on the date of this memorandum.

3. What this means in practice

Patients whose most recent potassium measurement falls within the new interval require no immediate action; they move onto the shorter schedule from their next measurement. Patients whose most recent measurement is older than the new interval should have one arranged.

The Committee wishes to be explicit on one point: this revision is not a statement that previously compliant care was substandard. It is a change to a threshold in light of new operating data, and it should be communicated to patients in those terms if the question arises.

4. Scope unchanged

The population remains as defined in SAFETY_DOC_007 - CardioZen recipients with a diagnosis of chronic kidney disease. This memorandum makes no change for patients without that diagnosis, and introduces no requirement for any test other than serum potassium.

5. Status of SAFETY_DOC_007

SAFETY_DOC_007 should be treated as superseded in respect of the monitoring interval in its section 3. Its remaining sections stand.

- Ends -